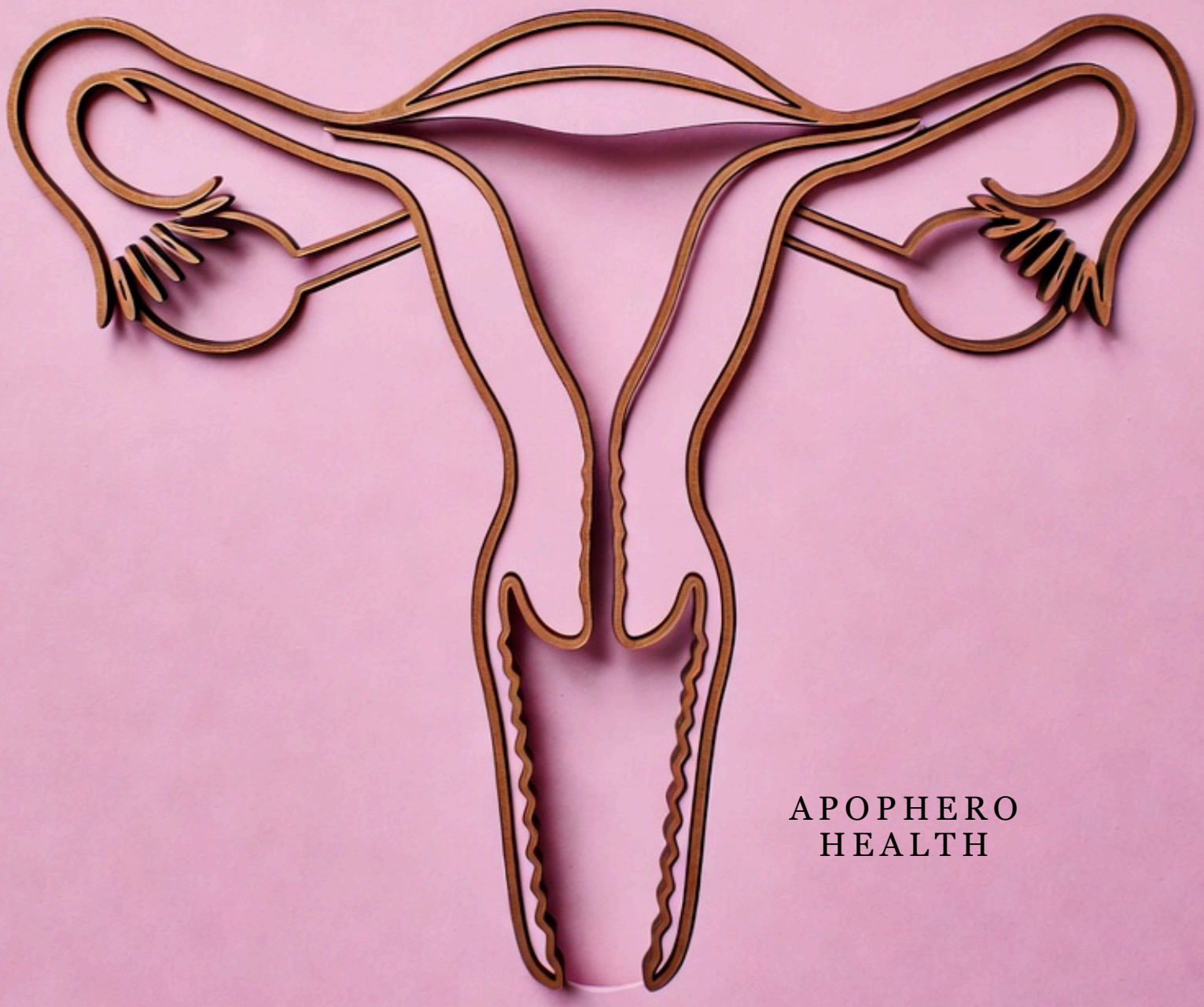
YOUR PERSONALIZED PCOS PREVENTION & REVERSAL PROTOCOL

POLYCYSTIC OVARIAN SYNDROME



APOPHERO HEALTH

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THE TRUTH ABOUT PCOS

Understanding PCOS

Polycystic Ovary Syndrome (PCOS) is the most common endocrine disorder in women of reproductive age, affecting 8–13% worldwide. A polycystic ovary has several FOLLICLES and in this case each ovary has more than 12 follicles. Polycstic ovaries have been called 'Oyster Ovaries' because they look like oyster shells (smooth, pearly, white surfaces).

WHAT REALLY HAPPENS IN PCOS

Ovarian follicles do not disintegrate in PCOS because of a decreased level of follicle-stimulating hormone (FSH) hence the ovarian cells cannot aromatize the androgens to estrogens, which leads to decreased estrogen levels and consequently leads to no ovulation.

An ovarian follicle is a fluid-filled sac that contains an immature egg. Follicles are found in the ovaries. During ovulation, a mature egg is released from a follicle. While several follicles begin to develop each cycle, normally only one will ovulate an egg. After ovulation, the follicle dies and turns into a corpus luteum. The follicles that do not release a mature egg disintegrate. About 99 percent of ovarian follicles will disintegrate and never become mature enough to release an egg but in PCOS they don't disintegrate.

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Major symptoms include

1.Menstrual dysfunction: Mostly dates back to teenage. Others have menstrual flow that occurs at intervals of 35 days to 6 months with less than 9 menstrual periods in a year or completely absent menstrual flow.

2.Hirsutism: Excess terminal body hair in a male distribution pattern. Hair on upper lips, chin, around nipples and lower abdomen. The clitoris is abnormally big and voice unusually deepens.

3.Infertility: Due to hormonal defects because of a dysfunctional (Hypothalamus - Pituitary - Ovarian) axis, women with PCOS mostly ovulate intermittently or fail altogether. Conception may take longer or primarily fail completely.

DIAGNOSITIC CRITERIA

You will be diagnosed with PCOS if you have in at least 2 of the following 3 criteria

1. Absent ovulation or reduced/ long intervals of menstruation
2. Clinical/biochemical evidence of high androgen hormones.
3. Polycystic ovaries on ultrasound (>12 follicles in an ovary)

4.Obesity: 50% of women with PCOS are clinically diabetic because PCOS is biochemically associated with insulin resistance and high insulin levels. Obese women with PCOS have abdominal girth with a waist circumference > 35 inches.

5.Diabetes / prediabetes: 50–70% of women with PCOS have insulin resistance.This means up to 1 in 2 will develop prediabetes or full type 2 diabetes by age 40 if nothing is done.

6.Obstructed sleeping patterns: Women with PCOS are 5–10 times more likely to have sleep apnoea than women without PCOS. Why?Because the high testosterone,central obesity and insulin resistance leads to relaxed throat muscles collapse at night which leads to snoring and choking, stopping breathing dozens of times per hour.

TREATMENT OF PCOS

The treatment for PCOS is divided into two, Lifestyle modification and Drug treatments. In this guide we focus on lifestyle modifications

1.Lifestyle modification

Such as diet and exercise is not “helpful” or “supportive”, it is the official first-line therapy for every woman with PCOS (according to the 2023–2025 International Evidence-Based Guideline).It actually involves lowering your insulin, moving your body daily and prioritizing sleep & stress management.

2.Drug treatments

Drugs are never first-line (official 2023–2025 guidelines). All drugs either mask symptoms or buy time, none cure the underlying insulin problem on their own meaning the symptoms return once you stop swallowing the drugs,that is why the fail.

- **Birth control pills:** Are given to create artificial regular bleeds and temporarily lower ovarian androgens for acne/hirsutism control.
- **Metformin:** Is given to directly lower insulin levels, improve insulin resistance and help restore ovulation in overweight women.
- **Spironolactone (or other anti-androgens):** Are given to block testosterone at the skin and hair follicles to reduce hirsutism, acne and scalp hair loss.
- **Letrozole or Clomiphene:** Are given to women actively trying to conceive to stimulate ovulation when lifestyle.
- **GLP-1 agonists (semaglutide, tirzepatide):** Are given for rapid weight loss and dramatic improvement in insulin sensitivity when obesity is severe.
- **Myo-inositol (or inositol combinations):** Are given as a natural insulin-sensitising agent to improve egg quality, ovulation rates and metabolic markers with almost no side effects.

Note.

Drug treatments are reserved for metabolic dysfunction such as poor ovulation, hirsutism and menstrual irregularities.

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THE TWO PROVEN PILLARS THAT ACTUALLY REVERSE PCOS

Lowering insulin is the only proven way to fix the root cause of PCOS and it is achieved through just two simple synergistic habits that I term as the two pillars of reversing PCOS.

Pillar one is DIET & EXERCISE: This involves eating a very low-carbohydrate (ketogenic) diet.

Pillar two is FASTING: This entails daily intermittent fasting or prolonged fasting depending on your capability.

When combined these two pillars rapidly restore ovulation, clear the skin and melt excess weight, this balances the hormones (Insulin inclusive) in the body within months without relying on medication therefore reversing PCOS in the long run.

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PILLAR ONE: DIET & EXERCISE

1.Ketogenic diet

This is eating a meal that has very low-carbohydrate, high-fat and moderate proteins. This forces the body into a metabolic state called **ketosis**. During ketosis, the body shifts from burning glucose for energy to burning fat for fuel producing ketones as an alternative energy source for the brain and body. This is achieved by drastically reducing carbohydrate intake, typically to fewer than 20-50 grams per day while increasing fat consumption to about 70%-80% of daily calories.

Why a ketogenic diet works.

- Lowers insulin
- Reduces testosterone
- Restores ovulation
- Reduces cravings
- Promotes weight loss

Foods to avoid (100% Insulin spikers)

- All added sugar & sweets.
- All grains & flours (wheat, rice, maize, millet, sorghum, oats, quinoa).
- Starchy vegetables (potato, sweet potato, corn, peas, butternut).
- Most fruit (except berries in small amounts).
- All beverages with calories (juice, soda, sweetened tea/coffee, alcohol).
- Processed/packed foods with hidden carbs.
- Artificial sweeteners that spike insulin (maltodextrin, sucralose in some people).

Foods to Eat Freely (Your new best friends)

- Eggs (any style).
- Fatty meat (ribeye, lamb, goat, pork belly, chicken thighs with skin).
- Fatty fish (salmon, mackerel, sardines, herring).
- All non-starchy vegetables (spinach, kale, broccoli, cauliflower, cabbage, zucchini, eggplant, mushrooms, asparagus, lettuce, cucumber).
- Healthy fats (butter, ghee, olive oil, avocado oil, coconut oil, MCT oil, tallow, duck fat)
- Full-fat cheese, heavy cream, Greek yogurt (unsweetened).
- Nuts & seeds (macadamia, pecans, brazil nuts, almonds (≤30 g/day), chia seeds, flaxseeds).
- Herbs, spices, salt, pepper, mustard, vinegar.

Concise PCOS reversal ketogenic weekly meal plan

Day	Meal 1 (12–2 pm)	Meal 2 (6–8 pm)	Daily Net Carbs	Notes
Monday	3-egg cheese omelette cooked in butter + ½ avocado	Grilled chicken thighs (skin-on) + sautéed spinach in garlic butter + 30 g cheese	~12 g	Easy start
Tuesday	Bulletproof coffee (black coffee + 1 tbsp butter + 1 tbsp MCT oil) + 2 boiled eggs	Fatty pork ribs or lamb chops + broccoli roasted in olive oil	~10 g	Higher fat day
Wednesday	3-egg scramble with 50 g halloumi + sautéed zucchini	Baked salmon with lemon-butter sauce + side of cauliflower mash with cream	~14 g	Omega-3 boost
Thursday	Leftover salmon + ½ avocado + handful macadamia nuts	Bunless cheeseburger patties (double) topped with bacon + side salad with olive-oil dressing	~11 g	Quick & satisfying
Friday	2 fried eggs + 4 strips bacon + sautéed mushrooms	Beef or goat curry cooked in coconut milk (no tomatoes/onions) + cauliflower rice	~15 g	Indian/Africa n flavours
Saturday	"Keto bomb" shake: heavy cream, unsweetened almond milk, 1 tbsp almond butter, ice	Tandoori or peri-peri chicken wings (skin-on) + creamy coleslaw (cabbage + mayo)	~12 g	Weekend treat feel
Sunday	-	Large ribeye steak or fatty fish + buttered asparagus + 50 g blue cheese	~10 g	Optional 22–24 h fast

Drinks & extras (unlimited)

- Water,sugarless black coffee,sugarless plain tea.
- Salt your food generously. Add pinch of salt in water if light-headed.

Note.

You can keep on changing the food based on it's availabity and test preferance. As long you maintain a daily net carb below 50g.
You can also consult us inacse you need a personalized meal plan.

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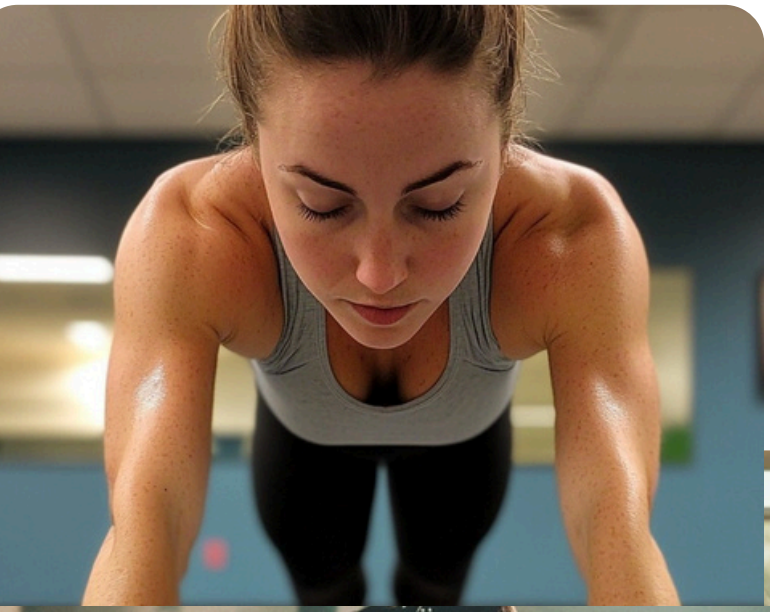
2.Exercise

Exercise is powerful medicine for PCOS because it directly improves insulin sensitivity (every workout acts like a temporary “metformin dose”) it lowers testosterone, reduces inflammation, protects fertility and speeds up fat loss especially the dangerous belly fat. The best workout for women with PCOS are resistance training and daily walking .

Note.
1.Avoid high intensity chronic cardio this is usually counterproductive because it raises cortisol and can delay periods.



Importance of exercise

It will minimize associated symptoms like diabetes, other metabolic disorders and features of masculinity.

Concise exercise weekly plan

Day	Type	Workout Details	Sets & Reps	Rest Between Sets
Monday	Rest	Rest	Rest	Rest
Tuesday	Full-Body	1. Goblet squats (dumbbell or kettlebell held at chest) 2. Hip thrusts 3. Dumbbell Romanian deadlifts 4. Seated Dumbbell shoulder press 5. Assisted Pull-ups	3–4 sets × 10–15	90 secs
Wednesday	Walking only	10,000+ steps (outdoors if possible)	-	-
Thursday	Rest	Rest	Rest	Rest
Friday		Light walk, yoga or gentle swimming (≤30 min)	3–4 sets × 10–15	90 sec
Saturday	Full-Body	1. Bulgarian Lunges 2. Dumbbell Bench Press 3. Single-Leg Glute Bridge 4. Dumbbell Rows (one arm) 5. Farmer's Carry (heavy dumbbells walk)	3–4 sets × 10–15	90 sec
Sunday	Rest or very light walk	Light walk, yoga, or gentle swimming (≤30 min)	-	-

You can always chip in new exercise like the ones below or the ones recommended by your gym trainer.

- Glute Bridges
- Standing Dumbbell Shoulder Press
- Lat Pull-down
- Bird-Dog
- Dead Bugs
- Pallof Press

PILLAR TWO: FASTING

Fasting can be **daily fasting** that is fasting for some hours of the day or it can be **prolonged fasting** for days inorder to archieve autophagy where the body cleanses itself.

1.Daily fasting is divided into two that is One Meal a Day **(OMAD)** or Two Meals a Day **(2MAD)**. It involves restricting eating windows to promote fat loss, metabolic health and potentially hormone balance.

- **OMAD:** Eat one meal within a 2-hour window, fasting for 23 hours. Here you eat between 3pm and 5 pm.
- **2MAD:** Eat two meals within a 8 hour window (16:8 fasting) then you fast for the rest of the day.

Whilst in any kind of fasting, you can take sugarless coffee or green tea in the morning. And also drink water to stay hydrated, you can add a pinch of salt.

Why fasting works.

- Dramatically lowers insulin
- Restores menstrual cycles
- Improves fertility
- Reduces weight
- Reduces inflammation
- Reduces ovarian cysts

2.Prolonged fasting is a strategy where you fast for a specific period to stimulate autophagy where old and harmful protein compounds are recycled for use whereas harmful plaques are being broken down .
Autophagy normally sets in at hour 18 of fasting,you can practice autophagy by fasting for two days nonstop.

Note.
Always break your fast with light food like bone broth, millet porridge, cabbage juice, carrot juice etc. Do not break your fast with bulk foods.

Here are some of the fasting schedules that you can start with.
If you are new to fasting I advise you start with the least hours.

Beginner (Weeks 1–2)

- 14:10-stop eating at 8 pm, first meal 10 am next day.

Intermediate (Weeks 3–6)

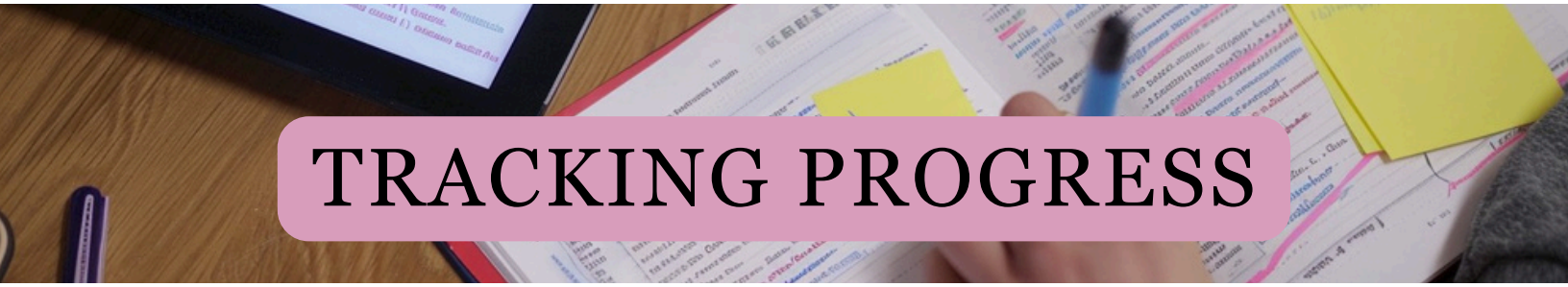
- 16:8-eating window 12 pm–8 pm (most popular & sustainable.

Advanced (Week 7+)

- 18:6 (1–7 pm window) or 20:4 (4–8 pm window).
- Optional: one 24–36 hour fast per week (dinner-to-dinner) to achieve good autophagy.

PCOS-Reversal fasting guide

Stage	Fasting hours	Eating hours	Best for	How to do it	When to move to next level
Beginner	14:10	10 am – 8 pm	First 7–14 days or if stressed/under weight	Last bite 8 pm → break fast next day 10 am	When you feel good & cravings are gone
Sweet spot	16:8	12 pm – 8 pm	Most women – fastest safe results	Black coffee morning → Meal 1 at 12–2 pm, Meal 2 by 8 pm	After 2–4 weeks
Accelerated	18:6	2 pm – 8 pm	When periods not back yet or stubborn weight	Meal 1 at 2 pm, Meal 2 at 7–8 pm → 18 h fast	When 16:8 feels easy
Advanced	20:4 ("Warrior")	4 pm – 8 pm	Very fast testosterone drop & fat loss	One big meal + small snack 4–8 pm only	After 1–2 months of 18:6
OMAD	23:1	One meal only (e.g., 6 pm)	Maximum insulin-lowering (many ovulate in 6–10 weeks)	Only one large keto meal per day	When body is 100% fat-adapted
Autophagy marathon	24–36 hours	Dinner-to-dinner	1–2× per week for massive reset	Finish dinner → water/tea only → next dinner	After 2–3 months on plan



TRACKING PROGRESS

Tracking is not about obsession or punishment, it is objective evidence that your ovaries are waking up, your insulin is dropping and your hormones are rebalancing. When you see the numbers and photos change month after month, motivation becomes automatic because you have undeniable proof that the plan is working even on the days you don't feel perfect.

1.Monthly tracking (Do this every 4 weeks – same day each cycle)

What to Track	How to Do It	Target After 3–6 Months on Plan
Menstrual cycle	Use Clue, Flo, or paper calendar	Regular cycles 25–35 days, ovulation signs (cervical mucus, BBT rise, positive OPK)
Weight	Same scale, same time (morning after toilet, naked)	5–10% loss is excellent; even 3–5% transforms hormones
Waist circumference	Measure at navel level, relaxed exhale	4–10cm decrease = massive insulin sensitivity improvement
Acne & hair growth	Take well-lit photos (face, chin, lower abdomen, chest/back) monthly	Visible clearing of inflammatory acne, slower/new hair growth
Energy, mood, sleep	Rate 1–10 in your app notes	Should climb steadily after week 4
Fasting glucose & insulin	Home glucometer + lab (fasting insulin + HOMA-IR if possible)	Glucose <5.0 mmol/L, insulin <8 mIU/L (ideal <5)

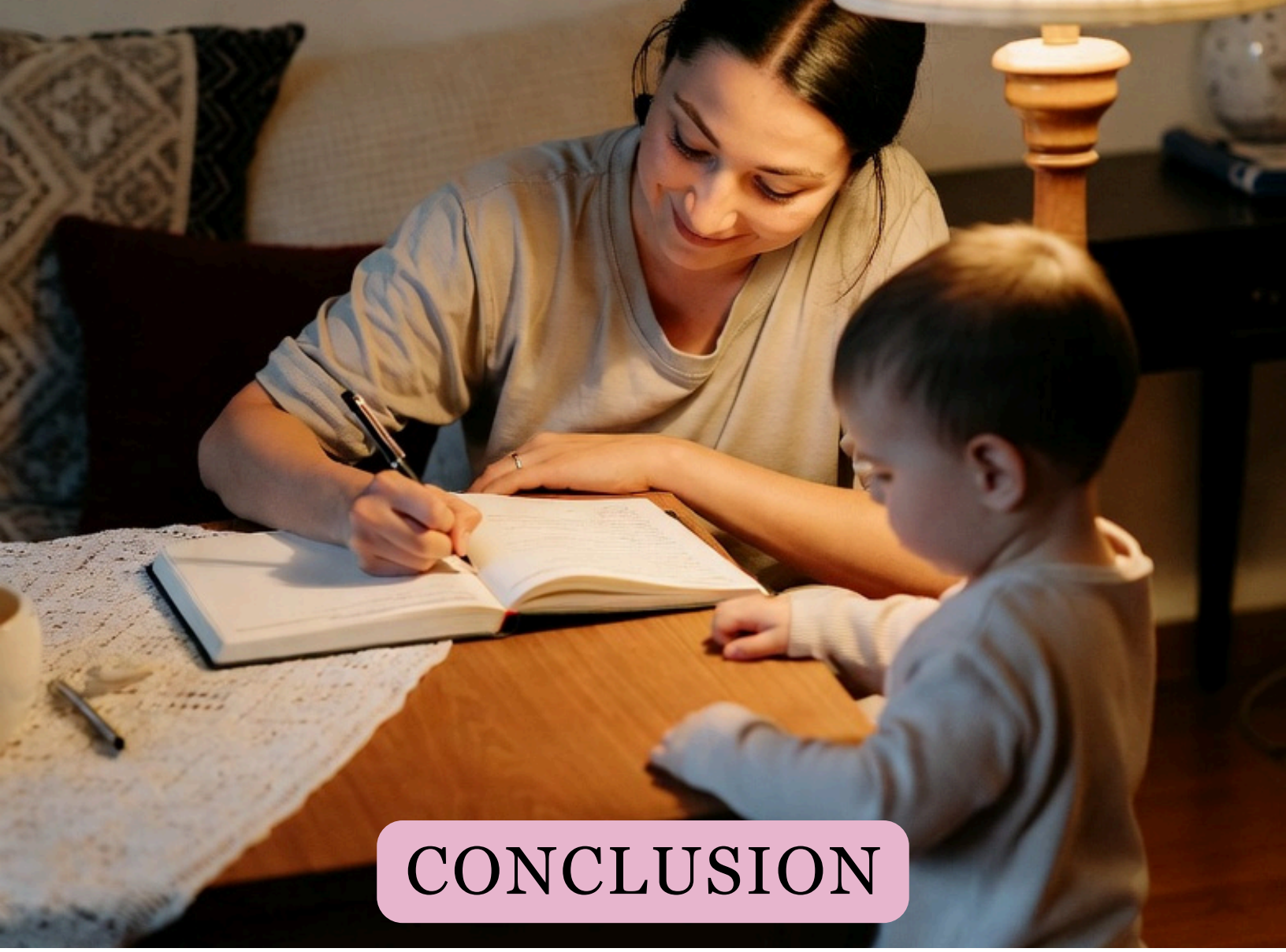
2.Big-milestone checks

When	What to Do	Why It Matters
Month 3	First do a bloodwork at the beginning then repeat at month 3: Testosterone (total + free), SHBG, LH/FSH ratio, fasting insulin, HbA1c, lipids	80% of women see testosterone drop >30% by now
Month 6–12	Repeat transvaginal ultrasound	Ovarian volume usually ↓ 25–50%, follicle count drops dramatically – visible proof of reversal

3. Red-flag rule – See your doctor immediately if:

- No period at all for >3 months while strictly following low-carb plus 16:8 fasting.
- Possible pregnancy, thyroid issue or (rarely) over-restriction.
- Sudden rapid hair loss, extreme fatigue, feeling cold or hoarse voice.
- Screen for hypothyroidism or adrenal dysfunction.
- Very heavy bleeding that soaks a pad/tampon in <2 hours or lasts >10 days.
- Severe new depression, anxiety or suicidal thoughts.
- Planning pregnancy (even if cycles are back).
- Get full preconception bloodwork + partner semen analysis.

One of the biggest differences between women who fully reverse PCOS and those who “try but never quite get there” is simple: **the first group measures.**
Because PCOS reversal isn’t a hope it’s a visible, measurable process and this is how you watch it happen in real time.



CONCLUSION

PCOS is not a disease you “manage” forever with pills.
For the majority of women, PCOS is a reversible metabolic condition caused by high insulin.You now have the complete evidence-based roadmap used successfully by thousands of women worldwide.
Start today. Your body wants to heal. You’ve got this.
I am with you on your journey.

END

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